

Gonadotropin-Releasing Hormone (GnRH) Analogues in Prostate Cancer

Original Effective Date: 9/1/2022

Revision Date(s): 9/1/2022; 11/7/2022; 7/5/2024; 7/1/2025; 4/1/2026

Review Date: 4/1/2026

Policy type: Medical Necessity

Line of business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
Preferred	Yes	Firmagon (degarelix)	J9155	Injection, degarelix, 1 mg
Preferred	Yes	Eligard(leuprolide acetate depot)	J9217	Leuprolide acetate (for depot suspension), 7.5mg
Non-Preferred	Yes	Lupron Depot (leuprolide acetate depot)	J1950	Injection, leuprolide acetate (for depot suspension), per 3.75 mg
Non-Preferred	Yes	Camcevi (leuprolide mesylate)	J1952	Leuprolide injectable, camcevi, 1 mg
Non-Preferred	No	Lutrate Depot (leuprolide acetate depot)	J1954	Injection, leuprolide acetate for depot suspension (lutrate depot), 7.5 mg
Non-Preferred	No	Trelstar (triptorelin)	J3315	Injection, triptorelin pamoate, 3.75 mg
Non-Preferred	No	Camcevi ETM (leuprolide mesylate)	J9003	Leuprolide injectable (camcevi etm), 1 mg
Non-Preferred	No	Zoladex (goserelin)	J9202	Goserelin acetate implant, per 3.6 mg
Non-Preferred	No	Lupron Depot (leuprolide acetate depot)	J9217	Leuprolide acetate (for depot suspension), 7.5 mg
Non-Preferred	Yes	Orgovyx (relugolix)	J8999	Prescription drug, oral, chemotherapeutic, nos

- Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents prior to approval of a non-preferred product, when clinically appropriate.
- Preferred product requirements apply to new starts only.
- Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
 - Clinical Pharmacology (Strong For); **OR**
 - Wolters Kluwer Lexi-Drugs® (Level A); **OR**
- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
 - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association, Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;
 - **Meeting abstracts and case reports are excluded from consideration;**
- Non-standard protocols may be approved based on unique clinical circumstances;
- Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network® (NCCN), or indication specific peer-reviewed literature;
- In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

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17. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). National Comprehensive Cancer Network, Inc. 2026. All rights reserved. Accessed March 11, 2026. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
11/7/22	Policy approved by P&T
7/5/2024	Annual Review, approved by P&T
7/1/2025	HCPCS Code Description Update (J1954) for 7/1/2025
3/16/2026	OH P&T annual review -
4/1/2026	Policy adopted by GEHA; document updated for universal accessibility. Addition and HCPCS update for NTM drug Camcevi ETM (J9003)